

Training home health aides to become peer coaches in mental health and wellness: The MINDSET Study

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Abstract

Home health aides (HHAs) experience high rates of stress, depression, and social isolation, which threaten their health and workforce sustainability. To address this, we adapted the evidence-based *Living Healthy* peer coach (PC)-led, CBT-based training program for HHAs and assessed its feasibility and acceptability. From February to July 2024 we conducted a mixed-methods, single-arm pilot study with English- and Spanish-speaking HHAs, in collaboration with the 1199SEIU Training and Employment Fund (TEF). Participants completed baseline and post-training surveys and an interview. Twelve HHAs from 12 unique home care agencies in New York, NY completed the training (mean age = 52.4 years, SD = 11.5; 92% female; 75% Black; 25% Hispanic). All were certified as PCs and reported satisfaction and confidence. Qualitative findings revealed increased confidence and motivation, improved health behaviors, barriers related to time, technology, and materials, and interest in future programs. The training program was feasible and acceptable.

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Data Availability Statement included at the end of the article

Keywords

home health, home health aides, mental health, long term care, peer coaching

Background

Home health aides and attendants (HHAs) are a rapidly growing workforce of trained and certified health professionals who are essential for the care of older adults at home (Reckrey et al., 2019). Current estimates suggest 70% of older adults, including those with multiple chronic conditions, prefer to stay at home as they age and to avoid nursing homes, a concept referred to as “aging in place” (AARP, 2018). To achieve this, they may require help from caregivers, along with HHAs (Sterling et al., 2020; Sterling and Shaw, 2019; Van Houtven et al., 2020).

HHAs face significant health challenges, including high rates of stress, depression, and social isolation. For example, 28% of HHAs report frequent mental distress compared to 13% of the general U.S. workforce (Brody et al., 2018; Sterling et al., 2021). This may be due to the combination of workplace stressors and HHAs’ marginalized status within the healthcare system. Healthcare staff experience emotional demands and role strain that influence coping and help-seeking behaviors (Chan et al., 2024). These stressors are compounded by structural marginalization—most HHAs are women from minoritized racial and ethnic groups who earn dismal wages and whose professional insights are rarely sought by the broader healthcare team (Bensson-Ravunnirath et al., 2023; Sterling et al., 2018). These challenges have been documented internationally, highlighting the emotional and occupational demands of HHA work (Cullen-Smith et al., 2025). Further contributing to their health challenges, a high proportion are underinsured and do not visit doctors regularly, which limits their access to quality mental health care (Silver et al., 2020). If left unaddressed, poor mental health among HHAs threatens their health and the sustainability of the workforce. Additionally, given the increasing demand for home-based care as the population ages, there is an urgent need for affordable, scalable interventions to improve HHAs’ mental health so they

are able to adequately support themselves and their patients.

One promising approach to addressing the mental health needs of HHAs is through peer coaching (PC), defined as “support from a person who has experiential knowledge of a specific behavior or stressor and similar characteristics as the target population” (Dennis, 2003). PC allows for mutual, non-hierarchical relationships between peers, where individuals learn from each other by sharing their experiences (Fisher et al., 2017; Gallant, 2003). Previous studies have shown that peer-based interventions are feasible, acceptable, and can enhance coping and well-being among healthcare workers, helping them manage stress while fostering social and emotional connections (Somerton and Jeffrey, 2025; Timmins et al., 2025). In randomized controlled trials (RCTs), PC programs have been successful at improving health behaviors including diet and exercise, as well as physiologic outcomes such as blood pressure and glucose control, overall cardiovascular health, and symptoms such as depressive symptoms and chronic pain among community-dwelling adults (Andreae et al., 2020, 2021; An and Song, 2020; Chinman et al., 2015; Thom et al., 2013). HHAs are enthusiastic about PC, and it has proven successful at bringing an often isolated workforce together (Poon et al., 2023), as well as improving health behaviors and outcomes including glycemic control among HHAs with diabetes (Crain’s New York Business, 2017; Olson et al., 2016). In our recent studies, HHAs with poor mental health expressed a desire for education and support through PC (Yanez Hernandez et al., 2024). In particular, HHAs valued a forum to receive support with their health through a trained peer who understood their unique job-related challenges (Yanez Hernandez et al., 2024). Yet, to date, no programs exist that train HHAs to become PCs and provide mental health and well-being support that many HHAs desire.

To address this, we adapted the evidence-based PC program, *Living Healthy* (Andreae et al., 2021), to the HHA workforce and developed a tailored PC training program specifically for HHAs. Herein we: (1) describe the adapted *Living Healthy* PC training program for HHA PCs and (2) test its feasibility and acceptability among a cadre of HHAs.

Methods

Overview, study setting, and community partnership

We conducted this mixed-methods, single-arm pilot study with HHAs who were interested in becoming PCs from February 28, 2024 to July 23, 2024 in collaboration with the 1199SEIU Training and Employment Fund (TEF), a labor management fund of the 1199SEIU United Healthcare Workers East, the largest healthcare union in the US (1199SEIU Funds, 2023). 1199SEIU TEF provides education and training to more than 55,000 HHAs who are employed by over 55 licensed and certified home care agencies in New York (1199SEIU Funds, 2022). The 1199SEIU TEF has a long-standing program of PC, although HHAs had not previously been trained in mental health, well-being, or the *Living Healthy* program for HHAs. The time-stamped protocol and study details can be accessed at <https://clinicaltrials.gov/ct2/show/NCT06071221> (Identifier: NCT06071221). The preregistration complies with ClinicalTrials.gov disclosure requirements and institutional research transparency policies.

Overview and adaptation of the *Living Healthy* PC program for HHAs

Details of the original *Living Healthy* Program are published elsewhere (Andreae et al., 2018, 2020, 2021). Briefly, it is an evidence-based health education program that incorporates principles of cognitive behavioral therapy (CBT), teaching participants to recognize and modify negative thinking and outcome expectations through self-monitoring, reflection, and

practice. The program has been tested and evaluated across four RCTs in the US, including populations demographically similar to HHAs (Andreae et al., 2020, 2021).

To adapt the intervention for the workforce, we first conducted a qualitative study with HHAs to understand their challenges pertaining to mood, stress, and achieving good health, on and off the job (Yanez Hernandez et al., 2024). Findings informed adaptation of the *Living Healthy* program for HHAs into eight lessons (Supplemental File 1), each incorporating CBT principles and relevant examples for home care workers. For example, the lesson on “Physical Activity and Health” discussed the benefits of physical activity, ways for a HHA to stay active while working with less-mobile clients, and how to overcome negative thinking related to physical activity and replace it with positive thinking. We maintained the original intervention’s theoretical underpinnings, including the use of the social cognitive theory (SCT), which emphasizes that behavior change occurs through the interaction of personal, behavioral, and environmental factors through personalized goal setting, motivational interviewing, and peer modeling (Andreae et al., 2020). Key SCT constructs incorporated into the intervention included self-efficacy, observational learning, behavioral capability, and outcome expectations. The peer coaching format promoted observational learning and peer modeling by allowing HHAs to practice skills with one another and share experience related to workplace stress and health behaviors. As such, study materials were developed to encourage self-reflection, SMART (Specific, Measurable, Achievable, Relevant, Time-bound) goal setting, and other strategies to reduce stress, improve mood and overall health. Like the original intervention, alongside the eight lesson plans, we created a PC manual to guide coaches in delivering the content, with structured talking points for each topic. In addition, we created a *Living Healthy* Activity Book and a *Living Healthy* Monitoring Calendar (Figure 1), to help HHA learners track their health behaviors and habits during the program, and for PCs

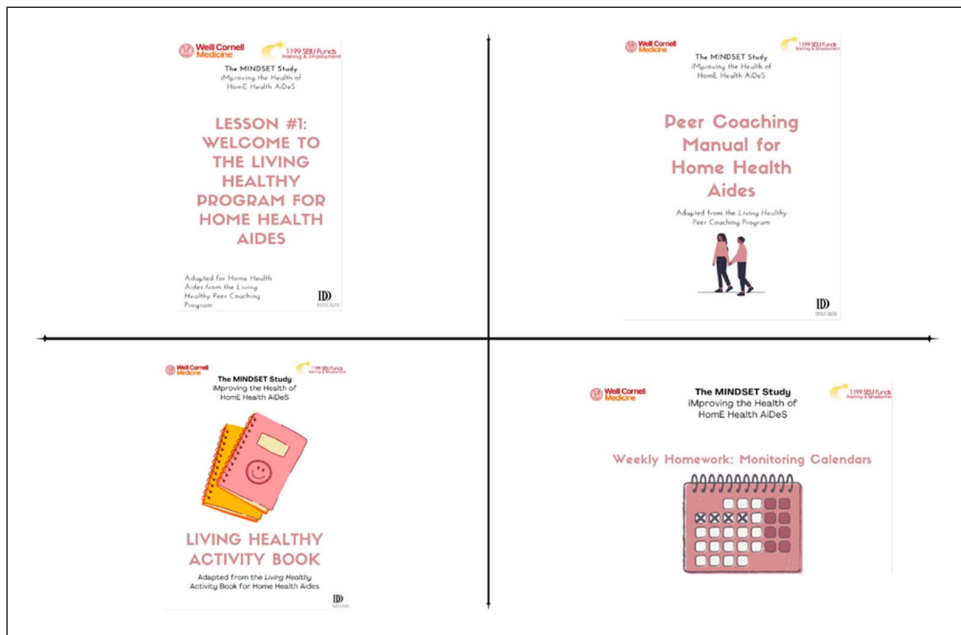


Figure 1. The MINDSET Study book sections. (1) *Living Healthy* lesson plans; (2) *Living Healthy* peer coach manual; (3) *Living Healthy* activity book; and (4) *Living Healthy* monitoring calendar.

to review and identify issues to guide them toward better mental health and well-being.

Recruitment of HHAs to become PCs

We recruited a cohort of HHAs who had a desire to become PCs for a future study. To be eligible, HHAs had to speak English or Spanish, be at least 18 years of age, be affiliated with 1199SEIU TEF, and have had some exposure to a PC program in the past. During the study period, 1199SEIU TEF staff used a standardized recruitment script and referred potential participants to the Weill Cornell Medicine research assistants (RAs) to be screened for eligibility. Eligible HHAs provided electronic informed consent through REDCap, a secure electronic data capture tool (Harris et al., 2009, 2019). The study used a hybrid format (virtually and in-person) and took place in New York, NY over a 12-week period. This study was approved by the Biomedical Research Alliance of New York (BRANY) Institutional Review Board (No. 23-12-744-380).

Training schedule and certification overview

The goal of the 12-week training program was for HHAs to master the *Living Healthy* program for HHAs, so they could deliver it proficiently to HHAs in the future. To work toward this goal, enrolled HHAs first attended an all-day in-person Kick-Off Event that oriented them to the study team and materials and provided training on how to deliver the *Living Healthy* program (Figure 2). This training included CBT principles, communication skills such as asking open-ended questions, giving affirmations, reflective listening and summarizing (OARS), motivational interviewing, and setting SMART goals. HHAs were then divided into pairs and practiced dyadic role-play to simulate PC with their HHAs. These pairings would be maintained throughout the 12-week training program. All training sessions and materials were delivered in English.

At the end of the kick-off day, HHA pairs were evaluated and certified on Lesson 1 using the

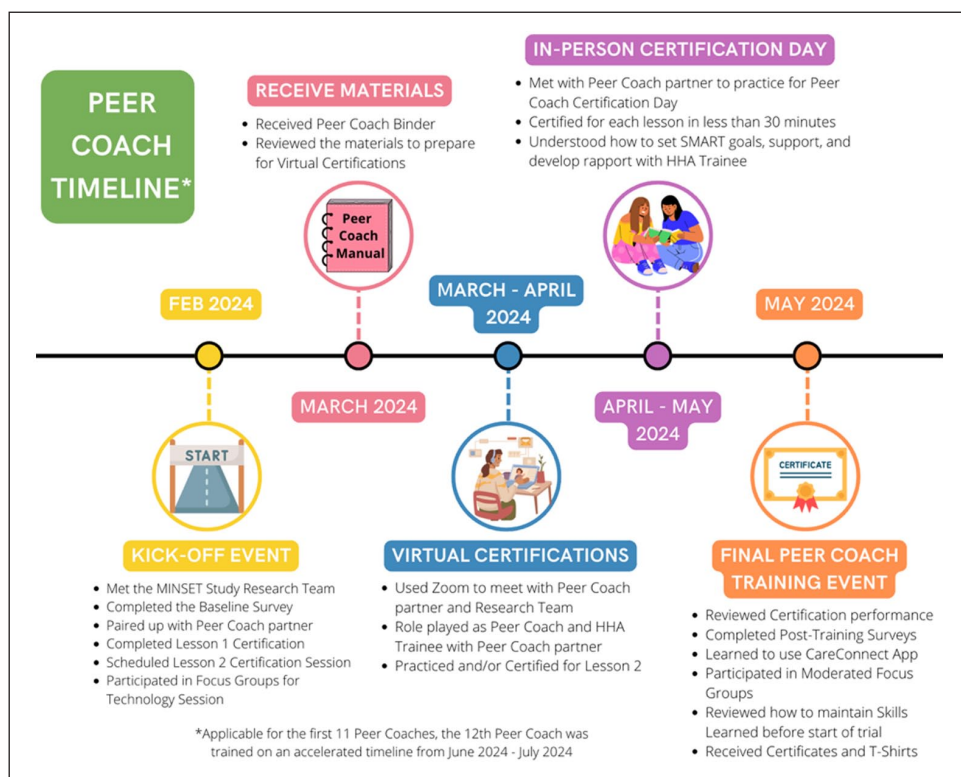


Figure 2. Peer coach timeline. Timeline of events for training and certifying peer coaches from kick off, receipt of materials, virtual and in-person certifications, to final peer coach training event (February 2024–May 2024).

Competency Checklist adapted from a previous study that assessed the feasibility and effectiveness of a virtual peer coach training program for individuals with rheumatoid arthritis (Brown et al., 2023). It evaluates the performance and technical proficiency of the lesson and ensures standardization of intervention delivery (i.e. fidelity). It is comprised of three sections: (1) Session Content, (2) OARS Evaluation, and (3) General Manner Assessment. The Session Content section was tailored to each lesson plan. The OARS Evaluation assessed whether participants asked Open-ended questions, offered Affirmation, demonstrated Reflective listening, and encouraged SMART goal setting. The General Manners Assessment (i.e. whether they were supportive and kind to their partner, etc.) was also used. Each of these items were rated as “Yes,” “Needs Improvement,” or “No.” The research team

observed every lesson, scored performance, and provided constructive feedback. Peer coaches practiced with their dyadic partner to correct problem areas and learned to make adjustments with real-time feedback. PCs were required to achieve a composite score of at least 85% to certify for each lesson (see Supplemental File 2).

Once certified in-person on Lesson 1, HHAs were encouraged to meet regularly with their HHA partner bi-weekly via Zoom to practice and prepare for certification in the remaining seven lessons. During these sessions, they reviewed the lesson principles, homework, and how they applied what they learned to their health. Once they felt confident, the HHA pairs scheduled virtual certification sessions with a member of the research team, who evaluated their performance using a checklist and certified them in each lesson. During the certification sessions, HHAs

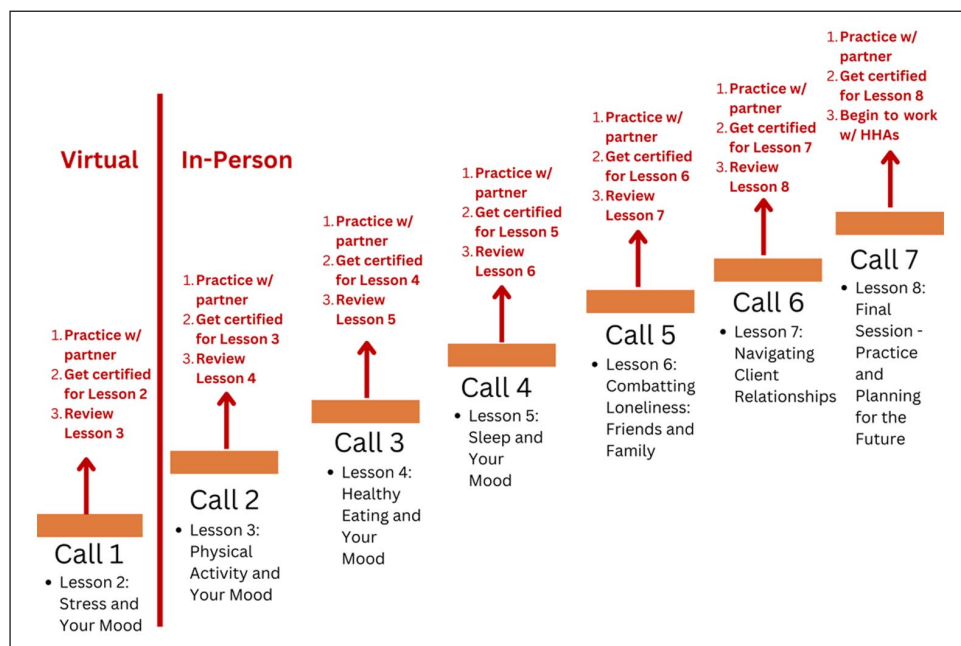


Figure 3. Peer coach certification breakdown (Lessons 2–8). Peer coaches transitioned from virtual to in-person certification between Lessons 2 and 3.

alternated roles as PC and HHA learner, to practice and demonstrate proficiency to be certified. Three research staff served as evaluators. Although certification was intended to be virtual, study staff recognized quickly (during Lesson 2 certification) that HHAs struggled with Zoom technology, and in particular certifying HHAs in a timely fashion while they followed along with corresponding study materials on screen proved to be difficult. To ensure timely certification, in-person certifications were conducted for all dyads for Lessons 3–8 (Figure 3).

At the conclusion of the 12-week training period, PCs attended a Final Training Event Day. During this all-day, in-person session, the research team shared certification results and reviewed strategies to maintain the skills they learned during their training period for the future.

Data collection

All participants completed a baseline survey prior to training that included demographic

characteristics (gender, age, race, Hispanic or Latinx ethnicity), education level, citizenship, primary language, and employment history. Additionally, data on employment attitudes, experience, skills, and confidence with PC, and comfort with technology were also collected. Most of these data were collected with novel Likert scales or 1-item novel questions, however, data from the Work Domain Satisfaction Scale (Bérubé et al., 2016), a 5-item validated scale, was used to assess job satisfaction, and the 2-item validated Intention to Leave Workforce Scale (Schaubroeck et al., 1989), were collected to assess turnover (Supplemental File 3). Following training, participants were asked to complete a post-training survey that assessed their experience in the program and confidence in serving as a PC in the future. They were also asked about the program's feasibility and acceptability using validated measures, the Feasibility of Intervention Measure (FIM) and the Acceptability of Intervention Measure (AIM) (Weiner et al., 2017). Their skills as a PC

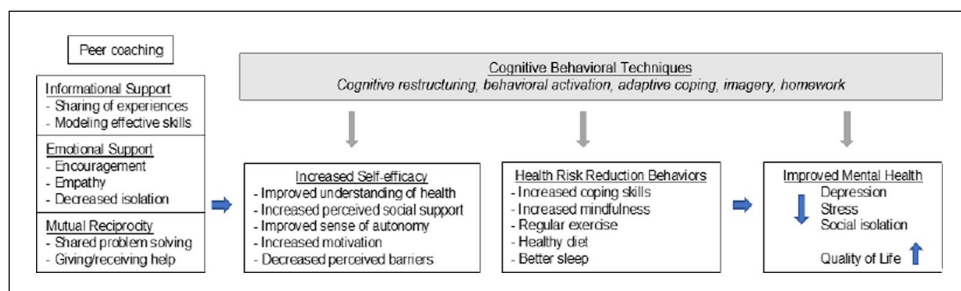


Figure 4. Adapted social cognitive theory framework. Adaptation of the social cognitive theory framework for behavioral interventions among home health aides.

were assessed using a novel 10-item scale on skill acquisition (Supplemental File 4).

In addition to quantitative assessments, HHA participants were also interviewed about their experiences by participating in focus groups (60 minutes in duration). Members of the research team moderated the focus groups using a semi-structured topic guide. The topic guide, informed by the Social Cognitive Theory (Figure 4), (Bandura, 1998) asked participants about (1) their perceptions of the *Living Healthy* Peer Coach Training program, (2) the content and quality of the *Living Healthy* program materials, (3) how challenging it was to complete the *Living Healthy* Peer Coach Training program, and (4) how confident and willing the training program made them feel in being a peer coach in a subsequent clinical trial.

Additionally, research team members who were not involved in the certification process conducted semi-structured 1:1 interviews with the research team members who served as Evaluators during the PC certification process to elicit their feedback and to further assess the feasibility and acceptability of the program. They were asked about their perceptions of the PC training program, including challenges encountered and how these were overcome, as well as benefits, and considerations for improvements in the future.

Compensation

Due to the number of hours PC training required, and in line with feedback and training compensation for the HHA workforce, each

participant was provided compensation for attending the baseline/kickoff event (\$100) and data collection, \$200 for completing the 12-week program, and \$100 for participating in the final day and focus group(s).

Data analysis

We performed descriptive statistics to characterize the sample (Carter et al., 2014; Morse, 2009). The distribution of the demographics, home care characteristics, readiness for Peer Coaching, comfort with technology and attitudes toward employment were analyzed for HHAs using percentages and means with standard deviations. Post-training, we calculated similar statistics to describe readiness for peer coaching, training experience, feasibility and acceptability. The study team compared select Peer Coach readiness measures pre-and post-training using a Wilcoxon signed rank test. All analyses were conducted using Stata/SE, version 18 software (StataCorp LLC).

Focus groups and interviews were audio recorded, professionally transcribed, and coded within Dedoose Software. We used a thematic analytic approach to develop a codebook with framework-developed a priori and emergent inductive codes (Braun and Clarke, 2006; Creswell, 2007; Curry et al., 2009; GitHub, 2024). Two investigators (R.P. and J.A.) independently reviewed the transcripts and created codes. The research team met to reconcile and consolidate their codes into an initial codebook, with assistance from two additional investigators

(M.S.) and (S.R.Y.), and oversight from a senior investigator (M.R.S.). The codes were categorized into themes and sub-themes by the research team (M.R.S., R.P., J.A., M.S.) with feedback from a qualitative research expert (S.R.Y.). Given the pilot design and small sample size, all quantitative and qualitative data analyses were exploratory, intended to identify preliminary trends to inform future program development and refinement.

Results

Participant characteristics

Our study cohort consisted of 12 HHAs employed by 12 unique home care agencies in New York, NY. Their mean age was 52.4 years (SD 11.5), 11 were female (92%) and 1 male (8%). Among them, nine participants identified as Black (75%) and three identified as Hispanic (25%). Overall, 7 (58%) spoke English as a primary language, and three spoke Spanish as a primary language (25%). The remaining two participants (17%) selected "Other" as their primary language. They had a mean duration of employment as a HHA for 13.7 years (SD 8.2) (Table 1). Four participants (33%) reported prior experience as a Peer Coach in a formal program, with a mean duration of 3.5 years (SD 1.7) in that role.

Quantitative data from study participants (HHAs)

HHAs reported different motivations for participating in the study and becoming a PC. As depicted in Table 2, the desire to learn about mental health was shared amongst all HHAs (100%), followed by the opportunity to learn about health in general (92%), and the ability to mentor others (92%). Notably, many HHAs were also motivated to participate so they could connect with other HHAs (83%).

In terms of PC readiness, all participants were confident in their ability to take on the role, with 33% feeling confident and 67% feeling very confident. With respect to skills

Table 1. Demographics and home care characteristics.

Characteristics	Value
N	12 ^a
Female	11 (92%)
Age, mean (SD)	52.4 (11.5)
Race	
African American Black	9 (75%)
White	1 (8%)
Other	2 (17%)
Hispanic	3 (25%)
Education	
Completed high school or GED	3 (25%)
Some college	2 (17%)
College degree	6 (50%)
Graduate degree	1 (8%)
Not born in United States	12 (100%)
Primary language	
English	7 (58%)
Spanish	3 (25%)
Other	2 (17%)
Years worked as home health aide, mean (SD)	13.7 (8.2)
Past experience as a peer coach	4 (33%)
Duration of past peer coaching role, mean years (SD)	3.5 (1.7)

^aHHAs were recruited from 12 unique agencies in New York, NY.

necessary for coaching, 42% strongly agreed that they work well with others, and 58% strongly agreed that they understood what it means to actively listen. Participants also showed strong goal-setting abilities, with 50% strongly agreeing that they know how to set goals for positive change. Despite this, confidence in their own leadership skills was mixed, with nearly 20% disagreeing or neutral, 17% agreeing, and 43% strongly agreeing (Table 2).

Regarding the *Living Healthy* program's value, most participants (84% and 91%) felt strongly or very strongly that they could apply what they learned to their clients and their family/friends, respectively. Additionally, 67% felt confident that they would be able to apply these skills in future experiences (Table 2). In terms of comfort with technology, participants reported

Table 2. Experience with peer coaching (baseline).

Characteristics	Value
N	12
<i>Motivation and readiness for peer coaching</i>	
What is your motivation for wanting to participate as a peer coach in this program ^a	
Learning more about mental health	12 (100%)
Opportunity to learn about health	11 (92%)
Opportunity to mentor others	11 (92%)
Wanting to connect with other HHAs	10 (83%)
Interest in participating in a research study	8 (67%)
Gift card for participating	2 (17%)
How confident are you in being a peer coach?	
Confident	4 (33%)
Very confident	8 (67%)
I know how to be a good mentor to others	
Neutral	3 (25%)
Agree	3 (25%)
Strongly agree	5 (42%)
Very strongly agree	1 (8%)
I work well with others	
Agree	4 (33%)
Strongly agree	5 (42%)
Very strongly agree	3 (25%)
I know what it means to actively listen	
Agree	2 (17%)
Strongly agree	7 (58%)
Very strongly agree	3 (25%)
I know how to set effective goals that help me achieve positive changes	
Neutral	1 (8%)
Agree	3 (25%)
Strongly agree	6 (50%)
Very strongly agree	2 (17%)
When it comes to leadership roles, I have no issues guiding others	
Disagree	1 (8%)
Neutral	1 (8%)
Agree	2 (17%)
Strongly agree	5 (42%)
Very strongly agree	3 (25%)
This program will teach me useful information that I can use with my family and friends	
Agree	1 (8%)
Strongly agree	4 (33%)
Very strongly agree	7 (58%)
This program will teach me useful information that I can use with my clients	
Agree	2 (17%)
Strongly agree	5 (42%)

(continued)

Table 2. (continued)

Characteristics	Value
Very strongly agree	5 (42%)
Participating in this program will be a good learning experience	
Agree	1 (8%)
Strongly agree	3 (25%)
Very strongly agree	8 (67%)
I will be able to apply skills learned in this program to other future experience	
Agree	1 (8%)
Strongly agree	3 (25%)
Very strongly agree	8 (67%)
I will be able to apply skills learned in this program to my job.	
Agree	1 (8%)
Strongly agree	4 (33%)
Very strongly agree	7 (58%)
<i>Technology as a modality for peer coaching</i>	
How comfortable are you with using Zoom or other video conferencing technologies	
Comfortable	5 (42%)
Very comfortable	7 (58%)
Please select all the technologies you use frequently (more than once a week) without assistance ^a	
Text messaging	12 (100%)
Voice calls	12 (100%)
WhatsApp	11 (92%)
Facebook/Facebook Messenger	10 (83%)
CareConnect	7 (58%)
Google chat	4 (33%)
Other	5 (42%)
Attitudes toward employment	
Job satisfaction, mean (SD) (range 17–30) ^b	24.3 (4.6)
I often think about leaving this job	
Very strongly disagree	1 (8%)
Strongly disagree	3 (25%)
Disagree	3 (25%)
Neutral	1 (8%)
Agree	4 (33%)
I will probably look for a new job outside of home care in the next year	
Very strongly disagree	1 (8%)
Strongly disagree	2 (17%)
Disagree	4 (33%)
Neutral	3 (25%)
Agree	1 (8%)
Very strongly agree	1 (8%)

HHA: Home Health Aide.

^aNot mutually exclusive categories.^bPossible range: 5–35.

being comfortable with video conferencing, with 58% feeling very comfortable using Zoom. They also reported frequent use of text messaging (100%), voice calls (100%), and WhatsApp (92%). Lastly, regarding attitudes toward employment the mean job satisfaction score was 24.3 (SD 4.6), indicating moderate satisfaction.

All 12 HHAs who participated in the PC program successfully completed it, with 100% certifying for all eight lessons. All 12 HHAs attended virtual and in-person certifications and certified on each lesson.

Following training, 42% and 58% of participants reported that the training program was positive or very positive, respectively (Table 3). Notably, after going through the training program, 75% of participants felt very confident to deliver the program as a PC to fellow HHAs, with 25% feeling confident.

The program was found to be feasible, with all peer coaches completing the training and achieving certification. The mean feasibility score was 4.5 (SD 0.3). All PCs completed the training and were certified, indicating strong skills, meaning the program was feasible. Specifically, 42% of participants agreed, and 58% completely agreed that the program could be implemented among HHAs. When asked about the possibility of coaching other HHAs, 58% agreed, and 42% completely agreed that they could effectively coach their peers toward better health. Half (50%) agreed, and the other half (50%) completely agreed that the materials were easy to use.

Regarding the program's acceptability, the mean acceptability score was 4.8 (SD 0.3). Specifically, 42% of HHAs agreed, and 58% completely agreed that the program met their approval as peer coaches and was appealing. Most HHAs (83%) completely agreed that they liked the *Living Healthy* program. Notably, all HHAs (100%) completely agreed with the statement, "I welcome the use of the *Living Healthy* program among HHAs" (Table 3).

With respect to skill acquisition notably, after training, there was a statistically significant improvement in "knowing how to be a good

mentor to others" (6 post-training median vs 5.5 pre-training median, p -value 0.02). Additionally, there were improvements across a variety of other domains, including "working well with others" (7 post-training median vs 6 pre-training median), "active listening skills" (7 post-training median vs 6 pre-training median), "goal setting for positive behavior change" (6.5 post-training median vs 6 pre-training median), and their "confidence as a leader" (7 post-training median vs 6 pre-training median) (Table 4). While these improvements in peer coaching skills are trending in the right direction, these differences did not reach statistical significance (Table 4).

Qualitative data from study participants (HHAs)

Guided by Social Cognitive Theory, our qualitative analysis of HHAs and Evaluators examined how peer coach training influenced key SCT constructs, including self-efficacy, behavioral capability, and behavior change, resulting in four key themes (Table 5).

Theme 1: Peer coach training enhanced HHAs' self-efficacy and behavioral capacity to coach peers and others. HHAs felt the *Living Healthy* PC training program enhanced their skills and confidence to coach their peers. Participating in this program helped them gain knowledge and communication skills that prepared them to support their peers. For example, Participant #3 from focus group 1 said,

"This experience has really taught me how to communicate more with my home care workers and to be able to impart whatever knowledge I have gotten from this peer coaching manual to them and to help them overcome any challenges they might have doing the job, and how to improve themselves."

Additionally, participating in the PC training program motivated them to deliver the *Living Healthy* program to their peers and beyond. Participant #1 from group 2 said,

Table 3. Peer coach training experience, feasibility of intervention and acceptability of intervention (post-training).

Characteristics	N (%)
N	12
<i>Training experience</i>	
Overall, how would you rate the training program experience?	
Positive	5 (42%)
Very positive	7 (58%)
How satisfied were you with the program content and study materials?	
Neutral	2 (17%)
Satisfied	2 (17%)
Very satisfied	8 (67%)
How was your experience working with a partner (peer coach partner) to complete your training?	
Neutral	1 (8%)
Positive	2 (17%)
Very positive	9 (75%)
After going through the program, how confident (or able and ready) are you in being a peer coach for the MINDSET Study?	
Confident	3 (25%)
Very confident	9 (75%)
<i>Feasibility of Intervention Measure (FIM)</i>	
The Living Healthy program seems like it could be implemented among HHAs	
Agree	5 (42%)
Completely agree	7 (58%)
The Living Healthy program seems possible for me to coach other HHAs on	
Agree	7 (58%)
Completely agree	5 (42%)
The Living Healthy program seems doable for Peer Coaches and HHAs	
Agree	8 (67%)
Completely agree	4 (33%)
The Living Healthy materials seem easy to use for HHAs	
Agree	6 (50%)
Completely agree	6 (50%)
FIM score, mean (SD) (range 4–5) ^a	4.5 (0.3)
<i>Acceptability of Intervention Measure (AIM)</i>	
The Living Healthy program meets my approval as a peer coach	
Agree	5 (42%)
Completely agree	7 (58%)
The Living Healthy program is appealing to me.	
Agree	5 (42%)
Completely agree	7 (58%)
I like The Living Healthy program	
Agree	2 (17%)
Completely agree	10 (83%)
I welcome the use of The Living Healthy program among HHAs	
Completely agree	12 (100%)
AIM score, mean (SD) (range 4.3–5) ^a	4.8 (0.3)

HHA: Home Health Aide.

^aPossible range: 1–5.

Table 4. Skills and readiness for peer coaching, pre and post training.

Motivation and readiness for peer coaching ^a	Pre-training median (IQR)	Post training median (IQR)	p-Value ^b
I know how to be a good mentor to others	5.5 (4.5–6)	6 (6–7)	0.02
I work well with others	6 (5–6.5)	7 (6–7)	0.09
I know what it means to actively listen	6 (6–6.5)	7 (6–7)	0.09
I know how to set effective goals that help me achieve positive changes	6 (5–6)	6.5 (6–7)	0.10
When it comes to leadership roles, I have no issues guiding others	6 (5–6.5)	7 (6–7)	0.08
This program will teach/taught me useful information that I can use with my family and friends	7 (6–7)	7 (7–7)	0.06
This program will teach/taught me useful information that I can use with my clients	6 (6–7)	7 (7–7)	0.12
Participating in this program will be/was a good learning experience	7 (6–7)	7 (7–7)	0.13
I will be able to apply skills learned in this program to other future experiences	7 (6–7)	7 (7–7)	0.31
I will be able to apply skills learned in this program to my job	7 (6–7)	7 (7–7)	0.36

^aPossible range for questions: 1–7.

^bp-Value from Wilcoxon signed rank test.

Table 5. Key themes of HHA's attitudes toward peer coach training.

Theme 1: Peer coach training enhanced HHAs' self-efficacy and behavioral capacity to coach peers and others

- Training built HHAs' coaching knowledge and skills, increasing confidence to coach others
- Training strengthened positive outcome expectations, motivating HHAs to coach beyond fellow aides

Theme 2: HHAs developed self-regulation and applied skills to their own health in real time

- HHAs identified health-related concepts and practiced strategies, demonstrating self-regulation in managing their health.
- HHAs engaged in positive health behaviors, reflecting their self-efficacy and behavioral capability
- HHAs reported early improvements in their own health, reinforcing positive outcome expectations

Theme 3: Although the program was positively received, environmental and structural barriers limited engagement

- Peer coach materials and manuals were environmental barriers to implementation
- Competing work demands and time constraints limited engagement

Theme 4: Desire for future peer coach programs tailored to address occupational challenges and support ongoing self-efficacy

- HHAs expressed enthusiasm for programs that bring peers together, supporting observational learning and outcome expectations
- HHAs emphasized the value of peer-based programs that foster community and mutual support through reciprocal determinism
- Programs should address occupational barriers that impact HHAs' health, emphasizing environmental facilitators and barriers

"It is working for me. So I can safely come to you and say, I would love to train you and it will improve your health because it works for me."

Many HHAs shared that the program inspired them to apply what they learned with family members and clients. Participant #1 from focus group 3 said,

"It's very beneficial because we're going to support our family with this. It's going to give them a healthy condition, our client too."

Evaluators echoed these observations. As one Evaluator noted,

"They're [the PCs] developing a sense of community, and they're developing the tools to expand that community by mentoring someone else."

Theme 2: Through the program, HHAs developed self-regulation and applied skills to their own health in real time. Participating in the program helped HHAs learn more about their own health and identify areas for self-improvement. Participant #2 from focus group 1 shared,

"I've incorporated walking now more than ever. I used to take vehicles everywhere. I don't like walking, none at all. But when I realized, no, seriously, I'm lazy when it comes to that. When I realized, when I started this, I said, I'm going to try."

Additionally, participant #4 from group 1 shared,

"I was able to identify and say, hey, this gluttony and eating too much, And I'm like, what can I do to change it? So, reading all those, it gave me the cause."

Not only were they able to identify areas of their health that needed improvement but also took action to support healthier behaviors. Participant #2 from focus group 2 said,

"I stopped buying sodas."

"I try to use less sugar."

Additionally, some HHAs reported early positive impacts of the program on their own health, reinforcing their motivation. Participant #3 from group 2 said,

"And I see a lot of changes and what I did was, like, last week I went for blood work...And I see there is a lot of progression, you know, positively."

Theme 3: Although the program was positively received, environmental and structural barriers limited engagement. While HHAs generally viewed the program positively, they did encounter challenges. One major challenge was the length of the *Living Healthy* book. Participant #3 from focus group 2 expressed,

"The book is voluminous. It has a lot of contents. . ."

Additionally, challenges arose about the format of the *Living Healthy* book, particularly with navigating between the lesson plans section of the book (trainee material) and the Peer Coach manual (peer coach material). Participant #3 from group 2 said,

"It's really kind of the going back and forth with the whole thing [from lesson plans to PC manual] is a challenge."

Evaluators highlighted this challenge as well. As one evaluator noted,

"If there were a way to organize it [Living Healthy book] such that they don't have to keep going back and forth."

Time constraint was another significant barrier. HHAs' demanding work schedules along with other responsibilities such as attending school, made participation difficult. Participant #2 from group 2 expressed,

“Limited time, work – we do a lot of things.”

Related to managing multiple responsibilities like work and school Participant #1 from focus group 3 said,

“There are little challenges, time management because there are a lot of things, you know, going to school, going to work and everything.”

Evaluators also highlighted the need to accommodate to HHAs work schedules was an ongoing challenge. As an evaluator said,

“Trying to get training that was accommodating to their schedule was one issue.”

Lastly, the training modality presented a challenge. In-person training was more effective due to technical challenges with video conferencing and greater engagement during in-person training. As another evaluator expressed,

“I think it became pretty evident early on that it was better to do it in person.”

Theme 4: Desire for future peer coach programs tailored to address occupational challenges and support ongoing self-efficacy. HHAs were enthusiastic about participating in future health education and PC programs due to the benefits they experienced. Participant #3 from focus group 2 said,

“I also believe that I can be able to take on any other program like this especially health wise programs. In the sense that it makes me more connect with people and also the knowledge as part of it because the more I do this the more I learn, the more I, you know, practice it.”

Participants found that the peer aspect of the program (training in pairs and in groups) fostered a sense of community and mutual support. HHAs expressed that being able to share challenges they faced on the job with a peer who shares similar experiences was particularly valuable. Participant #2 from group 1 explained,

“I think they should have more programs designed for home care workers to help each other because we go through so many challenges, and we can’t even talk to our supervisor. . .because the things we will discuss with other home care workers you cannot discuss with your coordinator.”

At the same time, HHAs emphasized that future programs need to address the occupational barriers they face, which make it challenging to manage their own health and coach others. These included time, balancing coaching with current shifts/schedules, and other commitments. Participant #3 from focus group 3 explained this with the following,

“Well, it’s really two things, sleep and healthy eating. And depends the hours you work, what is your shift like, you cannot really carry a solid meal that’s healthy for you. You just carry a snack, always the snack consists of bread, bread, bread, sandwiches. You know, you can’t carry something that you got to heat up. It’s always cold, even if you heat it at home before you go. So you find eating healthy is a big problem, especially for me, until I get home in the evening.”

Additionally, HHAs highlighted that their job is uniquely stressful as they are solely responsible for the well-being of their client, making peer coaching another responsibility on top of that which would need to be balanced. For example, participant #1 from focus group 4 shared,

“We are under stress. I believe my work, our work is double of the stress because we are responsible of everything that happens with the client in their house. We are behind doors. Nobody else is with us. If an emergency happens, we are responsible of the life of that person.”

Discussion

This study aimed to train HHAs to become PCs for the *Living Healthy* for HHAs program and evaluated their readiness to serve as PCs, as well as the program’s feasibility and acceptability. All 12 HHAs who began the training

program completed it and were certified as PCs. Participants reported increased confidence in coaching peers and applying health-promoting behaviors in their own lives. Qualitative interviews revealed that the training motivated them to adopt healthier behaviors and support others, including patients and their personal networks, in making positive health changes. The peer-based design allowed participants to observe and learn from one another, practice communication and mentoring skills, and share challenges unique to home care. Having peers who understood their work was especially valuable in addressing social isolation and fostering motivation. These findings align with prior studies demonstrating that peer-based interventions can improve coping, social support, and health behaviors among healthcare workers (Somerton and Jeffrey, 2025; Timmins et al., 2025). Our findings indicate that the adapted *Living Healthy* program was both feasible and acceptable to PCs to deliver to their fellow HHAs in a future program. This is especially timely given the home care workforce shortage crisis and the need for innovative and scalable programs that support HHA well-being and retention.

Our findings support the growing body of evidence on the positive impact of PC for health behavior change, particularly among vulnerable groups of patients and healthcare workers (Andreae et al., 2020, 2021; An and Song, 2020; Chinman et al., 2015; Thom et al., 2013). However, ours is the first to rigorously train and evaluate HHAs as PCs in the context of mental health, a particularly challenging issue in the home health care environment, which can often be isolating. Prior programs have indeed shown success, including a diabetes management program for HHAs that utilized peer coaching (Crain's New York Business, 2017), as well as a program that demonstrated the feasibility of using peer coaching to promote mobility among stroke patients (Feldman et al., 2022). Similar to prior studies with community health workers, our findings demonstrate the power of the reciprocal nature of PC, as well as CBT, goal setting, and motivational interviewing as key components that participants reported made

them better coaches while simultaneously promoting their own behavior change (An and Song, 2020; Fisher et al., 2017; Thom et al., 2013).

One notable finding was that HHAs expressed many motivations for participation, which may signal that this workforce could be leveraged to serve as PCs for a variety of topics and issues. For example, while many craved information and training on mental health, others relished the opportunity to serve as a mentor. For some, simply having a peer who understood their challenges provided a needed social support system that is often lacking when providing care to older adults in the home, highlighting peer coaching's potential in enhancing both personal and professional well-being. Taken together, our findings suggest PC may be a promising approach not only to improve HHA and patient health but also to promote workforce retention (Bensson-Ravunniarath et al., 2023; Sterling et al., 2021).

Our findings suggest, however, that future PC-led programs must be tailored to account for HHAs' unique challenges. Time constraints (due to HHA schedules), the organization of the *Living Healthy* program book, and technology-based training modality were significant barriers. Although HHAs initially reported feeling comfortable with video conferencing in the baseline survey, our study team recognized that in-person training was required for successful training and certification. Future programs should consider these structural and environmental barriers when designing training.

Strengths and limitations

A strength of our study is that we partnered with a labor-management fund of the largest health care union in the U.S., which allowed us to recruit a diverse sample of HHAs from various employment agencies. We used an established conceptual model to inform our interview topic guide and qualitative data analysis. Our study also has limitations. First, the small sample size limits statistical power and generalizability and findings should be interpreted as descriptive and exploratory. Still, this work

provides important insight into the feasibility and acceptability of an intervention aimed at addressing the mental health of the HHA workforce. Second, while we recruited a diverse sample of HHAs, we did not include nonunionized or non–agency-employed HHAs or HHAs who lived outside of New York City, which further limits generalizability. Finally, the use of virtual certification sessions introduced logistical challenges and required a concerted effort to coordinate; in contrast, in-person certification for multiple lessons was more efficient and easier to arrange.

Conclusion

We were able to train and certify a cadre of HHAs to become PCs to deliver the *Living Healthy* for HHAs program. The training was feasible, acceptable, and effective in enhancing skills and confidence of participants (PCs) and their engagement with health and self-reported health behaviors. HHAs expressed strong satisfaction with the program and were committed to seeing it implemented among fellow HHAs, with minor modifications to its delivery. Overall, our findings demonstrate a successful coaching program and pave the way for future PC-led programs amongst this workforce, which have the potential to improve the health and well-being of HHAs and enhance the care they can provide their patients.

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Ethical considerations

The study protocol was approved by the Biomedical Research Alliance of New York (BRANY) Institutional Review Board (IRB #23-12-744-380).

Consent to participate

All participants provided written informed consent prior to participation, signifying their understanding of the study objectives, potential risks and benefits, confidentiality measures, and their right to withdraw at any time without penalty. All experiments were performed in accordance with relevant guidelines and regulations.

Consent for publication

Consent for publication is not applicable to this article as it does not contain any identifiable data.

Author contributions

M.R.S., M.M.S., S.J.A., F.W., F.S. contributed to the study conception and design. R.P., M.S., J.A., M.R.S., J.R. drafted the initial manuscript. M.S., R.P., K.C., M.M.S., S.J.A., I.N.M., C.L., F.W., F.S. contributed to study materials. R.P., J.A., M.S., Y.E., K.C., J.S., A.S., I.R.S.K., contributed to recruitment and data collection. R.P., J.A., M.S., M.R.S., J.R., and S.R.Y. contributed to data analysis. All authors critically revised the manuscript for important intellectual content and approved the final version.

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Data availability statement

The data that support the findings of this study are available from the corresponding author, M.R.S., upon reasonable request.*

Supplemental material

Supplemental material for this article is available online.

Preregistration

Research was preregistered in the independent, institutional registry ClinicalTrials.gov (NCT06071221) on September 29, 2023 (<https://clinicaltrials.gov/ct2/show/NCT06071221>) with an analysis plan.

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